Week 1 report

Annex A | Project scoping and review protocol | 13 November 2026

Repository: https://github.com/FarahHammamii/FibroRag_Week1

What I did

I defined the scope, prepared a PCC review protocol and a search strategy for medical and technical literature. I ran the pilot locally on three databases and used the results to refine the query selection. I also prepared the M0-M3 comparison and eight draft FibroidQA scenarios. No assistant or model experiments have been completed yet.

Design decision

I chose evidence-aware selective RAG over a standard RAG-only project or a knowledge graph. Standard RAG becomes my M1 baseline; a graph would require too much medical validation. The aim is to isolate the effect of evidence ranking, verification and abstention.

Results

12/12 queries succeeded; 347 records exported; 296 unique; 51 duplicates removed. The archived run passes 28 hash checks, and two deduplication unit tests pass. The title labels are exploratory; formal screening has not started.

Difficulties

The AI environment could not reach the APIs, so I ran the search script locally. Broad queries returned too many records, and the Tunisian search did not establish a national prevalence. Some near-duplicates and full texts still need checking.

AI use

I used ChatGPT to explore three project directions, prepare Python search code, and organize the experimental design. I used actual database counts instead of estimates, kept the standard baseline separate, and limited the novelty claim after finding related work.

Self-assessment

Apply: I ran 12 database queries. **Analyze:** I compared the pilot volumes and title relevance patterns. **Evaluate:** I chose the narrower query strategy and kept uncertain records for checking. **Create:** I structured a testable M0-M3 research plan with ablations and draft benchmark scenarios.

Next week

1. Freeze the protocol and screen titles/abstracts. 2. Check full texts, quality and source licenses. 3. Start the evidence corpus and gold questions. Target: a documented screening log and initial corpus plan.